



Mental health consumers who are missing or absent without leave

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Original endorsement date	September 2003
Date of current endorsement	October 2025
Functional sub group	Mental Health
Summary	This policy articulates the processes to be followed when a consumer of a South Metropolitan Health Service mental health service is missing / absent without leave or is suspected to be missing / absent without leave from an inpatient unit or an Emergency Department or is reported missing from their usual place of residence in the community.
Policy owner	Executive Director Mental Health
Contact	Carole Steiner
Review date	13 October 2028
NSQHS Standards	Standard 1: Clinical Governance Standard 5: Comprehensive Care Standard 6: Communicating for Safety
Chief Psychiatrists Standards for clinical care	Standard 2: Assessment Standard 3: Care planning Standard 4: Consumer and carer involvement in individual care Standard 6: Risk assessment and management.
Status	Active

1. Background

1.1. About this document

This policy establishes the minimum requirements for reducing the risk of a mental health consumer becoming missing or absent without leave (AWOL) and ensuring the safety of mental health consumers who are missing or AWOL. The policy applies to mental health consumers in the inpatient or community setting and Emergency Departments (ED).

South Metropolitan Health Service (SMHS) Mental Health will follow site-based protocols and procedures to inform and guide staff in the event a consumer is found to be missing or AWOL from an area other than a mental health inpatient unit.

Refer to:

[FSFHG Missing Inpatients](#)

[RkPG Missing or suspected missing patients](#)

[PHC Missing or suspected missing Patients](#)

1.2. Key definitions

Term	Definition
Absent consumer	A mental health consumer whose whereabouts should be known but cannot be ascertained after diligent efforts to locate and/or contact them have been made by staff; however, their life or health is not believed to be in danger, and they are not believed to be at risk of harming others.
Absent without leave (AWOL)	When a person who is detained or involuntary is absent without approved leave from a hospital or other place Mental Health Act 2014 (MHA 2014) s. 97.
Involuntary patient	A person who is under an involuntary treatment order, either: • an inpatient treatment order or • a community treatment order MHA 2014 s. 21.
Mental health consumer	A consumer who has had contact with, or been treated by, public mental health services, including mental health services provided in EDs.
Missing person	Defined by the Chief Psychiatrist as 'Any voluntary psychiatric patient at high risk of harm who is missing from a mental health service, general hospital or emergency department, without the agreement of or authorisation by staff.' 'Harm' includes harm to self, harm to others and harm to property, such as arson.
Personal Support Person	Can be any of the following people who are supporting someone who is experiencing mental illness – close family member, carer, their nominated person; the parent or guardian of a child or any enduring guardian of an adult MHA 2014 s.7. Some consumers may use the term kin to refer to a family member.

Voluntary patient

A person who is proposed to be, or is being treated, by a mental health service voluntarily.

2. Policy Statement

SMHS Mental Health manages mental health consumers who are missing or AWOL in accordance with the [MHA 2014](#) and the [WA Health Mental health consumers who are missing or absent without leave policy](#).

SMHS Mental health services must use a person centred, recovery oriented, culturally competent, trauma informed and developmentally appropriate approach when providing care to mental health consumers.

3. Risk mitigation strategies

Minimum risk mitigation strategies for all mental health consumers include:

- Providing a physically and culturally safe and welcoming environment
- Ensure Aboriginal consumers are offered contact and/or support from Aboriginal Mental Health Liaison (AMHL)/Aboriginal Health Practitioners (AHP)/Aboriginal Health Liaison Officers (AHLO)
- Orientating the consumer to the environment and the service
- Early assessment and reassessment of the consumers risk of going missing or AWOL
- Completing risk assessments in partnership with the consumer and their carer or other personal support person wherever possible
- The use of alerts for all consumers at risk of going missing or AWOL
 - alerts are to be placed in the consumers paper medical record (e.g. inpatient file), digital medical record and PSOLIS profile for all consumers with a history of, or assessed as at risk of going missing or AWOL
- Partnering with the mental health consumer, their carers or other personal support person, in the planning and delivery of the consumer's care across all stages of their care unless the consumer declines to consent to their involvement.

3.1 Inpatient setting

Additional risk mitigation strategies may include:

- Provision of an appropriate, clean and welcoming environment that ensures adequate privacy
- Facilitating access to cultural support including AMHL or appropriately experienced AHP/AHLO
- Use of interpreter services
- Considering the individual needs of consumers from culturally and linguistically diverse and LGBTQIA+ communities and consumers with disabilities
- Early decisions by the treating psychiatrist and multidisciplinary team about the type of leave available to the consumer

- Appropriate communication with the mental health consumer, their carers or other personal support person to inform and engage them in care planning and treatment options, length of stay and decisions about leave.
- Requesting the voluntary consumer inform staff of their whereabouts if they plan to leave the ward or ED
- Maintaining an appropriate level of patient observation
- Access to therapeutic activities
- Encouraging consumers to let staff know if they have concerns about being on the ward or in the ED
- Targeted nursing time for those at risk
- Promotion of safe access to home and managing consumers concerns about home
- Use of a sign in and out book
- Supporting and facilitating contact with carers or personal support persons and community connections
- Careful breaking of bad news
- Post incident debriefing

Refer to:

[WA Health Statewide Standardised Clinical Documentation for Mental Health Services](#)
[SMHS Care Coordination in Mental Health Policy.](#)
[SMHS Granting and Management of Leave for Mental Health Inpatients Policy.](#)

4. Inpatients

All inpatients must have a current risk assessment documented in their medical record which clearly identifies the assessed risk of the consumer going missing or AWOL.

Sites will identify and implement individualised preventative measures based on best practice and discussion with the consumer, carer or personal support person wherever possible to minimise the occurrence of consumers going missing or AWOL.

These individualised strategies will be documented in the medical record using the Treatment Support and Discharge Plan (webPSOLIS) and a Safety Plan where appropriate.

Inpatient staff will use clinical handover processes to communicate the consumers risk of going missing or AWOL and specific observation requirements for each consumer.

Refer to:

[WA Health Statewide Standardised Clinical Documentation for Mental Health Services](#)
[WA Health Clinical Handover Policy](#)

Where a consumer on escorted ground access or leave attempts to abscond, staff will refer to:
[SMHS Granting and management of leave for mental health inpatients policy](#)
[FSFH Patient leave and escort – mental health](#)

4.1. Actions when an inpatient is suspected of being missing or AWOL

Inpatient staff must take reasonable steps to always know the whereabouts of mental health consumers. If a staff member (regardless of discipline) suspects that a consumer is absent from the ward they will immediately inform the relevant shift coordinator / team leader.

Staff will determine the level of risk to the mental health consumer and others. This risk assessment will determine the extent of efforts made by staff members to locate the consumer.

Diligent immediate efforts to locate a consumer include but are not limited to:

- attempts to contact the consumer by phone.
- thorough search of the inpatient unit and environs
- review of Closed-Circuit Television (CCTV) footage as a potential means of locating the consumer on site.
- asking the carer or personal support person for information on the consumers possible whereabouts

The consumer's treating doctor or duty doctor is to be informed when it is reasonably suspected or has been confirmed that the consumer is absent. The treating team is responsible for responding in a timely manner according to the assessed nature and level of risk.

In deciding on a course of action, the treating team will consider:

- The consumer's status under the [MHA 2014](#).
- The consumers mental state at the last point of contact
- The consumers last assessed level of risk to self, to others and from others as documented in their medical record
- Assessment of risk and determining whether the consumer meets the definition of a missing person or AWOL
- Collateral information from the carer or personal support person.

The personal support person is to be informed of the patient's unauthorised absence at the earliest time possible.

Refer to:

[WA Health Best Practice Guideline: Mental Health Consumers Who are Missing or Absent Without Leave](#)

4.2. Consumer considered "at risk" and location unknown

When a consumer who is missing or AWOL and at-risk staff must take appropriate actions to ensure their safety.

Clinical risk should be determined at the time of the individual going missing and identified risk management strategies should be documented in the consumers' medical record. PSOLIS alerts are to be updated to communicate missing status should the consumer present at another service.

Where appropriate, community mental health clinicians and Aboriginal mental health workers may visit the consumers' residence to determine the consumers whereabouts and wellbeing.

A psychiatrist will make an Apprehension and Return Order [MHA 2014](#) Form 7D for consumers who are AWOL if satisfied that no other safe means of ensuring the consumer returns to hospital is reasonably available. Every effort should be made to return the consumer to hospital before an apprehension and return order is made.

Feedback to the carer or personal support person should occur at regular intervals to update regarding status of service actions as the situation changes.

After hours a notification is to be made to the Mental Health Emergency Response Line (MHERL) and relevant Mental Health Crisis Centre or Assessment and Treatment Team (ATT).

Continued efforts will be made to contact the consumer and will be documented in the consumer's medical record.

During this period whilst the consumer is AWOL, they retain the legal status of an involuntary patient under the [MHA 2014](#) who is AWOL until they are located and reviewed by the consultant psychiatrist or the [MHA 2014](#) form authorising their detention expires.

If the consumer remains missing or AWOL they may be discharged from the inpatient unit until such time as they are located and returned to the unit. Returning missing or AWOL consumers whose bed is occupied will be conveyed to the ED of the respective inpatient mental health service.

Refer to:

[WA Health Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy](#)
[Statewide Mental Health Bed Access, Capacity and Escalation Policy.](#)
[WA Health Road based Transportation for Mental Health Consumers Policy](#)
[SMHS Transportation of Mental Health Consumers Policy](#)

4.3. Involving WA Police

Notification must be made to WA Police Force where there is a genuine and serious concern for the consumers welfare and safety and/or there is suspicion of criminality surrounding their disappearance.

When communicating with WA Police in relation to a missing person or AWOL consumer, staff members must:

- clearly communicate any risk of harm to the mental health consumer and/or others
- recognise the terminology used by WA Police Force and ensure mutual understanding of the situation
- keep WA Police apprised of any changes in risk
- document actions in the consumer's medical record

Refer to:

[WA Health Procedures for involving police when mental health consumers are missing or absent without leave.](#)

4.4. Absent consumers considered not at risk

An attempt is to be made to contact the consumer to try to determine their whereabouts.

A decision may be taken to discharge a voluntary patient who is not considered to be a risk to themselves or others.

The carer or personal support person should be informed that the patient has absented themselves from the unit and that they are to be discharged from hospital.

Staff are not required to notify WA Police where a mental health consumer is neither AWOL nor a missing person.

4.5. Safe return of patient

When a consumer who is missing or AWOL is located and returned by mental health service staff, or returns to the inpatient unit themselves, the shift coordinator / team leader will ensure that:

- A formal identification check is conducted immediately on return.
- The consumer is treated respectfully
- Staff will listen to and respond to any concerns raised by the consumer
- The mental health consumer's carers or personal support person are notified
- External stakeholders including WA Police are notified if appropriate
- Where staff hold a reasonable concern that the consumer is in possession of inappropriate or dangerous items a search will be conducted
- There is a discussion with the consumer (and with their personal support person or culturally appropriate appointed support person, if possible) to determine reasons why they went missing or AWOL
- The consumer's Risk Assessment and Management Plan is updated to document identified risk factors and management strategies to help prevent future AWOL or missing person incidents
- A physical health check and mental state assessment of the returning consumer is conducted and documented
- Treatment and care are reviewed with the consumer and the Treatment, Support and Discharge Plan should be updated to reflect any changes
- Risk alerts are updated in PSOLIS.

Refer to:

[SMHS Patient search and identification of dangerous items](#)

5. Mental health patient missing from ED

For a mental health consumer in a SMHS Emergency Department, staff will follow local procedures and protocols.

In the case of a mental health consumer in ED who is on a Referral for Examination by a Psychiatrist [MHA 2014](#) Form 1A an Apprehension and Return Order [MHA 2014](#) Form 7D may be completed where clinically indicated.

Refer to:

[FSFHG Mental Health Patient Management: Emergency Department](#)

[RkPG Missing or suspected missing patients](#)

[PHC Missing or suspected missing patients](#)

6. Community mental health consumers

All community mental health consumers must have a current risk assessment documented in their health record.

Consumers receiving mental health services in the community may be absent from their usual place of residence or fail to attend scheduled appointments for a number of reasons regardless of their status under the [MHA 2014](#).

If a consumer is absent from their residence during a scheduled home-visit or they fail to attend a scheduled appointment or activity, the community mental health staff member, must consult with the treating team to determine what action should be taken.

- When it is determined by the treating team that the absence is of concern the staff member is to take appropriate steps to obtain information on the whereabouts and wellbeing of the consumer.
- All steps taken will be documented in the consumer's medical record.

Refer to:

[WA Health Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy](#)

[FSFHG Did not attend: Outpatient appointments.](#)

[RkPG Mental Health Consumers who Do Not Attend Community Appointments: Management of](#)

6.1. Involuntary patient in community

A consumer being managed on a Community Treatment Order (CTO) may be required pursuant to terms of order [MHA 2014](#) s. 115 to comply with directions about where and when treatment is to be given [MHA 2014](#) s. 117.

Where a consumer who is subject to a CTO fails to attend a scheduled appointment or is absent from home for a scheduled visit, the supervising psychiatrist must be notified as soon as practicable.

Following discussion with the treating team, actions are to include some or all of the following:

- Visit(s) to the consumer's usual residence
- Phoning them on their mobile phone number if known
- Contacting personal support person
- Making discreet enquiries of neighbours
- Informing MHERL/ATT
- Notifying WA Police Force where there is a genuine and serious concern for the consumers welfare and safety and/or there is suspicion of criminality surrounding their disappearance.
- Admitting the patient to hospital.

If a breach of a CTO occurs staff must comply with the [MHA 2014](#) s. 127. The supervising psychiatrist must write a record of the breach and give notice of the breach to the involuntary mental health consumer detailing what the consumer must do to comply.

The actions of the supervising psychiatrist are to be guided by the CTO provisions of the [MHA 2014](#) s.113-125, taking into account details of the treatment plan.

A breach of CTO must be documented in compliance with the [Mental Health Regulations 2015](#)

Refer to:

[MHA 2014](#) s. 126-131 Breach of Order

[WA Health Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy.](#)

7. Incident reporting and investigation

A consumer who is missing or AWOL is a notifiable incident and must be notified as a clinical incident in Datix Clinical Incident Management System (CIMS). The CIMS entry will specify the Severity Assessment Code (SAC) based on the patient's risk status immediately prior to their absence.

The HoS / Service Director is responsible for informing the SMHS MH Executive of any SAC1 missing or AWOL consumer and again when the patient is located / returned.

Where possible, investigations of missing or AWOL mental health consumers should include consideration of lessons learned from post return discussions or interviews with the consumer and their carer or personal support person. Considering a consumer or carers account of a SAC1 incident may offer additional strategies to reduce recurrence.

Refer to:

[Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)

[WA Health Clinical Incident Management Policy](#)

[SMHS Mental Health Clinical Incident Management Procedure](#)

8. Roles and Responsibilities

8.1. Executive

SMHS MH Executives will ensure managers and supervisors are aware of the [MHA 2014](#) and DoH mandatory policy requirements relating to mental health consumers who are missing or AWOL.

8.2. Managers/supervisors

Managers / supervisors will:

- Establish and maintain a physically and culturally safe and welcoming environment that ensures adequate privacy
- Ensure mental health staff complete Aboriginal Person-Centred Care training
- Ensure staff conduct early assessment and timely reassessment of the risk of a consumer going missing or AWOL and develop and implement risk management strategies
- ensure that staff complete risk assessments in partnership with the mental health consumer, their carers/family and personal support person, and other agencies involved in their care, where possible and appropriate

- Ensure staff are provided with appropriate training and professional development in relation to missing person and AWOL incidents including:
 - Risk assessment and safety planning
 - Reducing risk and appropriate escalation of care.
 - Developing and maintaining therapeutic relationships

8.3. Staff

Staff will:

- Consider the likelihood of a consumer becoming AWOL or a missing person in risk assessment and safety planning
- Implement individualised preventative strategies based on best practice to minimise the occurrence of consumers going missing
- Engage with mental health consumers and where appropriate carers and support persons in the planning and delivery of the consumers care
- Make appropriate efforts to locate a consumer prior to further action being taken
- Escalate care appropriately where it is determined the consumer is missing or AWOL.

9. Compliance monitoring

9.1. Compliance monitoring

Each SMHS MH site or Service Director/Co-Director is to ensure compliance with this policy. Compliance monitoring methods include the following:

- Investigation of all missing person and AWOL incidents
- Annual thematic review of all reported missing person and AWOL incidents

10. Legislation

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

- [Mental Health Act 2014](#)
- [Mental Health Regulations 2015](#)
- [Carers Recognition Act 2004](#)

11. Related Documents

11.1. WA Health mandatory policies

- [MP 0012/16 Mental Health Consumers Who are Missing or Absent Without Leave Policy](#)
- [MP 0015/21 State-wide Standardised Clinical Documentation for Mental Health Services](#)
- [MP 0122/19 Clinical Incident Management Policy](#)
- [MP 0063/17 Road based transportation for mental health consumers Policy](#)
- [MP 0099/18 Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy](#)

- [MP 0181/24 Safety Planning for Mental Health Consumers Policy](#)
- [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- [MP 0095/18 Clinical Handover Policy](#)
- [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#)

11.2. SMHS policies

- [SMHS Care Coordination in Mental Health](#)
- [SMHS Granting and Management of Leave for Mental Health Inpatients](#)
- [SMHS Deaths Reportable to the Coroner](#)
- [SMHS Transportation of Mental Health Consumers](#)
- [SMHS Mental Health Clinical Incident Management Procedure](#)
- [SMHS Patient search and identification of dangerous items](#)

11.3. Other documents

- [WA Health Best practice guideline mental health consumers who are missing or absent without leave](#)
- [WA Health Procedures for involving police when mental health consumers are missing or absent without leave](#)

12. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP V7	23 October 2025	15 October 2025	15 October 2028	October 2028	Triennial Review – Name change, formally known as Missing or suspected missing mental health patients

12.1. Approval

Approval by	Executive Director Mental Health
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This document can be made available in alternative formats on request.

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